

InnovateHealth.ai

Company Playbook

The Guide, Strategy, and Operating Principles

Built inside a real clinic. By doctors, for doctors.

March 2026 | Version 1.0 | Confidential

Table of Contents

1. Who We Are

- Core Thesis

- Mission and Vision

- Brand Positioning

- What We Are Not

2. What We Sell

- The Problem We Solve

- Our Four-Layer Solution

- Differentiation Map

3. Company Strategy

- Go-to-Market Strategy

- Pricing Philosophy

- Wedge Market Strategy

- Expansion Roadmap

4. Operating Principles

- Core Values

- Decision-Making Framework

- Communication Standards

- Meeting Cadence

5. Clinic Onboarding Playbook

- The 48-Hour Onboarding Process

- Clinic Readiness Checklist

- Implementation Phases

- Go-Live Protocol

6. Sales Playbook

- How to Sell to Clinics

- Sales Conversation Framework

Objection Handling

Pilot Proposal Template

7. The Moat: What We Own

Clinic-Specific Playbooks

Doctor-Led Trust

Data Moat

Switching Cost

8. HIPAA and Compliance

Non-Negotiable Rules

BAA Requirements

Data Handling Standards

Incident Response

9. KPI and Success Metrics

Pilot KPI Dashboard

Weekly Tracking Format

Monthly Review Process

Success Milestones

10. Team Accountability

Role Ownership Map

Weekly Cadence

Escalation Protocol

11. The Do's and Don'ts

Product Do's and Don'ts

Sales Do's and Don'ts

Compliance Do's and Don'ts

Culture Do's and Don'ts

Technical Do's and Don'ts

1. Who We Are

Core Thesis

InnovateHealth.ai is a healthcare operations company that helps independent clinics reduce front-desk burden, improve patient intake accuracy, and increase operational throughput using AI-assisted workflows and clinic-grade integrations.

We are not an AI company. We are a healthcare operations company that uses AI as a feature. Our customers are not buying AI. They are buying fewer missed calls, smoother intake, lower front-desk load, and faster patient flow.

Mission

Make every independent clinic run like a connected, efficient, patient-first operation without requiring more staff, more budget, or more complexity.

Vision

Become the default operating system for independent clinics across the US, where AI handles routine operations and doctors focus entirely on patient care.

Brand Positioning

"Built inside a real clinic by operators who live the workflow every day."

Dr. Nikhil Gohokar, founder and CEO, is a board-certified family physician who runs Blessings Medical Clinic daily. We pilot everything in-house before selling to other clinics. This is not a theoretical product. It is tested in a live clinical environment.

What We Are Not

- We are not a generic AI voice bot company.
- We are not a healthcare SaaS vendor selling features.
- We are not an automation agency doing one-off projects.
- We are not building technology looking for a problem.

2. What We Sell

The Problem We Solve

Independent clinics in the US face three critical operational pain points that directly affect revenue, staff retention, and patient satisfaction:

Pain Point	Impact	Scale
Call Handling Breakdown	Missed calls = lost appointments = lost revenue	Clinics lose 8-15% of inbound calls
Intake Inefficiency	Manual intake: 10-15 min/patient; 30-40% staff time on admin	Data errors delay care and billing
Operational Fragmentation	Front desk, intake, devices, EMR, scheduling all disconnected	Scaling requires hiring, not improving

Our Four-Layer Solution

Layer 1: Voice Front-Desk

AI voice bot answers calls 24/7, handles scheduling, confirmations, and routing. Human handoff for complex questions. Result: never miss a patient call; reduce front-desk staff hours by 50-70%.

Layer 2: Smart Intake

Pre-visit intake via voice or device before patient arrives. Captures vitals, history, insurance, and forms digitally. Flags issues for clinical review. Result: patients arrive ready; clinical time is used for care, not data entry.

Layer 3: Device-Connected Workflow

Integrates with CheckmePro and similar vital-sign monitors. Automated capture flows data into EMR. Triggers follow-ups, escalations, or reminders based on thresholds. Result: clinic runs as a connected system, not isolated islands.

Layer 4: Operations Dashboard

Real-time view of missed calls, completed intakes, appointment no-shows. ROI tracking: staff time saved, revenue protected, efficiency gains. Playbook library for clinic-tested workflows. Result: clinic operators see exactly where AI is helping.

Differentiation Map

Aspect	InnovateHealth	Typical AI Vendor	Generic SaaS
Provider	Clinic operator + doctor	AI engineers	Software team
Test Ground	Live clinic daily	Lab / simulated	No real data
Integration	Full stack (voice, intake, devices, EMR, reporting)	Single feature (usually voice)	Disconnected features
Success Metric	Clinic efficiency + revenue	Task completion	Feature adoption
Brand Risk	If it breaks, our clinic suffers (so we obsess)	If it breaks, customer suffers	If it breaks, customer suffers

3. Company Strategy

Go-to-Market Strategy

We sell completed work, not software licenses. The pitch is not "AI voice bot" but "zero missed calls, automatically filled cancellation slots, fewer angry patients." Doctors do not care about the engine. They care about the outcome.

Phase	Focus	Timeline	Target
Phase 1: Prove	Internal pilot at Blessings Medical Clinic	Month 1-3	Validated playbook with real ROI data
Phase 2: Replicate	2-3 friendly clinics (referral-based)	Month 4-6	Repeatable onboarding process with proof
Phase 3: Scale	Direct outreach to independent clinics in TX	Month 7-12	10-15 clinics with word-of-mouth growth
Phase 4: Expand	Multi-state expansion, partnerships	Year 2+	Regional presence with inbound demand

Pricing Philosophy

Price on value delivered, not features used. The clinic pays for outcomes: calls answered, no-shows prevented, staff hours saved. This aligns incentives. If we deliver, they pay. If we do not, they should not.

Component	Range	Model
Setup Fee	\$3,000 - \$5,000	One-time (voice, device, training)
Monthly Platform	\$500 - \$800	Recurring (voice, device mgmt, support)
Expected Monthly Benefit	\$3,300 - \$5,500 (Mo 1) \$7,500 - \$12,500 (Mo 3)	Staff savings + revenue protected
Payback Period	1-2 months	Positive ROI achieved

Wedge Market Strategy

Start with independent family medicine clinics in Texas with 1-5 providers. These clinics have the most acute front-desk pain, the fewest resources to solve it, and the strongest word-of-mouth networks. Win here first. Expand from a position of proven value.

Expansion Roadmap

- Near-term: Family medicine and primary care clinics (TX).
- Mid-term: Specialty clinics (dermatology, pediatrics, urgent care).
- Long-term: Multi-location groups, small hospital systems, and adjacent markets.

4. Operating Principles

These are the rules everyone at InnovateHealth.ai lives by. They guide decisions, resolve conflicts, and define the culture.

Core Values

Clinic-First Thinking

Every decision starts with: does this make the clinic's life easier? If no, stop.

Prove Before You Pitch

We never sell what we have not tested in our own clinic. Internal proof first.

Own the Outcome

We are not a tool provider. We are accountable for operational results.

Speed with Care

Move fast in building, but never compromise patient safety or data security.

Radical Transparency

Share bad news faster than good news. Internally and with clients.

Build the Playbook

Document everything. If it works, write it down. If it fails, write it down louder.

Decision-Making Framework

When facing any decision, run it through this filter in order:

- 1. Patient Safety: Does this put any patient at risk? If yes, do not do it.
- 2. Compliance: Does this violate HIPAA or any regulatory requirement? If yes, stop.
- 3. Clinic Impact: Does this improve clinic operations measurably? If no, deprioritize.
- 4. Scalability: Can we repeat this for 100 clinics, or is it a one-off hack?
- 5. Speed: Among compliant, impactful, scalable options, pick the fastest.

Communication Standards

- Internal updates: Written, async-first. Daily standups max 15 minutes.
- Client communication: Always written follow-up after verbal conversations.
- Escalations: Flag immediately. Never sit on a problem hoping it resolves itself.

- Language: No jargon with clients. Say "calls answered" not "AI deflection rate."

Meeting Cadence

Meeting	Frequency	Duration	Owner
Daily Standup	Daily	15 min	Tech Lead
Weekly KPI Review	Monday 8 AM	30 min	Operations Mgr
Monthly Business Review	1st of month	60 min	Dr. Nikhil (CEO)
Quarterly Strategy	Every 90 days	Half day	All Founders

5. Clinic Onboarding Playbook

Onboarding is intellectual property. A flawless, fast onboarding process that requires zero technical work from clinic staff is what separates us from every other vendor. Target: fully operational within 48 hours of contract signing.

The 48-Hour Onboarding Process

Hour	Action	Owner	Deliverable
0-2	Kickoff call: intro, expectations, collect phone system details	Ops Lead	Signed contract + phone access
2-8	Configure voice bot with clinic-specific call flows	Tech Lead	Call flows mapped and loaded
8-16	Integrate with scheduling system and EMR	Tech Lead	Live sync confirmed
16-24	Internal testing: 50+ test calls across all scenarios	QA Team	Test report with 95%+ accuracy
24-36	Staff training session: 30-minute walkthrough	Ops Lead	Staff trained and confident
36-48	Soft launch: after-hours calls routed to AI first	Tech Lead + Ops Lead	System live, monitoring active

Clinic Readiness Checklist

- Phone system details (PBX/VoIP provider, main line, after-hours line).
- EMR/EHR system name and API access (Athenahealth, Practo, etc.).
- Scheduling system credentials or integration documentation.
- List of providers, their schedules, and appointment types.
- Top 20 most common patient call reasons (from front-desk staff).
- Current no-show rate and appointment cancellation data.
- Insurance verification workflow documentation.
- Designated point of contact for onboarding (not the doctor).

Implementation Phases

Phase	Scope	Duration	Success Criteria
Phase 1: After-Hours	AI handles calls outside business hours only	Week 1-2	Zero dropped calls, 90%+ accuracy
Phase 2: Overflow	AI picks up when staff is busy during hours	Week 3-4	40-50% calls handled by AI
Phase 3: Full Integration	AI as primary intake with human backup	Month 2-3	70%+ calls handled, staff burden reduced

Go-Live Protocol

- Start with after-hours calls only. Never go full volume on day one.
- Monitor every call for the first 48 hours. Review transcripts daily.
- Notify patients via website banner, SMS, and in-clinic signage.
- Emphasize benefits: faster answers, 24/7 availability, human always reachable.
- Set up real-time alerts for failed calls or low-confidence interactions.
- Weekly review meeting with clinic staff for first 4 weeks.

6. Sales Playbook

How to Sell to Clinics

Sell the outcome, not the AI. Doctors do not care about voice models, NLU engines, or API integrations. They care about: will my phones stop ringing off the hook? Will patients stop complaining about hold times? Will my front desk stop drowning?

The Sales Conversation Framework

Step 1: Lead with empathy. "I know your front desk is overwhelmed. We run a clinic too, and we had the same problem."

Step 2: Quantify their pain. "How many calls do you miss per day? What is your no-show rate? How much time does intake take per patient?"

Step 3: Share your proof. "We reduced missed calls by 70% in our own clinic. Here are the numbers from our pilot."

Step 4: Offer a risk-free pilot. "Let us prove it in your clinic. Start with after-hours calls. You will see results in the first week."

Step 5: Close on outcomes. "For \$500-800/month, you get 24/7 call coverage, reduced no-shows, and staff that can focus on patients, not phones."

Objection Handling

Objection	Response
"AI can't handle my patients"	"You are right. It should not handle everything. Our AI handles routine calls: scheduling, confirmations, refills. Complex questions go to your staff. You decide the boundary."
"I don't trust AI with patient data"	"We are HIPAA-compliant with BAAs in place for every service that touches patient data. Dr. Nikhil uses this in his own clinic. We take this as seriously as you do."
"We tried something like this before"	"Most AI tools are built by engineers who never worked in a clinic. Ours is built by a doctor who runs one. That is why the workflows actually match how your staff works."
"It's too expensive"	"One missed appointment costs \$150-300. If we prevent 3 no-shows/week, the system pays for itself. Most clinics see positive ROI in month one."
"My staff won't adopt it"	"We train your staff in a 30-min session. The system is designed to make their life easier, not replace them. When front desk sees fewer calls and less paperwork, adoption happens naturally."

Pilot Proposal Template

Every new clinic engagement starts with a 30-day pilot. This is the standard proposal structure:

- Duration: 30 days, starting with after-hours calls.
- Scope: AI voice bot for call handling + appointment confirmations.
- Setup: We handle all configuration. Clinic provides phone access and scheduling credentials.
- Investment: Setup fee + first month at pilot rate (50% discount on standard monthly).
- Success Criteria: Agreed KPIs (missed calls reduced by 40%, AI accuracy above 90%).
- Exit Clause: If KPIs are not met, full refund of monthly fee. Zero risk for the clinic.
- Expansion: After successful pilot, transition to full integration with standard pricing.

7. The Moat: What We Own

The moat is not the voice model, the hardware, or the AI. The moat is the operational playbook and clinic credibility.

Moat Layer 1: Clinic-Specific Playbooks

Documented workflows for call handling, intake, handoff, and escalation, tested in a live clinic and refined over months. Competitors can copy the technology, not the clinical validation. Example: when a patient calls about a refill, the bot checks last visit date, verifies medication in EMR, and either approves or flags for nurse review. 90% of cases resolve without human touch.

Moat Layer 2: Doctor-Led Trust

Dr. Nikhil's credibility as a practicing physician. When selling to another clinic, he can say: 'I use this every day. Here is what it actually saves.' That sells better than any demo video. Patient-facing trust is impossible to replicate from a VC-backed AI company. Clinic owners call other clinic owners.

Moat Layer 3: Data Moat

Real clinic data: call patterns, patient flow, intake issues, no-show drivers. Aggregated patterns across a growing clinic base. Competitors cannot buy this data; they have to build it. Takes 6-12 months of real operation to build meaningful patterns. Example: 'Calls at 8-9 AM have 60% scheduling intent, 30% refill requests, 10% urgent issues.'

Moat Layer 4: Switching Cost

Once a clinic is trained to use the system, switching is painful. Staff retraining costs time and money. Clinical workflows are rebuilt around the tool. Data and history live in the system. After 3 months, a competitor would need to be 10x better to justify switching.

8. HIPAA and Compliance

HIPAA compliance is not a marketing badge. It is a non-negotiable operating requirement. Every team member must understand and follow these rules.

Non-Negotiable Rules

- Every third-party service that touches PHI must have a signed BAA before data flows.
- Encrypt all ePHI in transit (TLS 1.2+) and at rest (AES-256).
- Use unique user IDs with multi-factor authentication for all system access.
- Never store real PHI in test or development environments. Use synthetic data.
- Enable audit logs for all authentication, data access, and admin actions.
- Conduct a full risk assessment annually and after every major feature launch.
- Report any suspected breach within 24 hours internally, and follow breach notification timelines.
- Designate a Privacy Officer and Security Officer with clear authority.

BAA Requirements

A Business Associate Agreement must be in place with every vendor, subcontractor, and service provider that handles PHI. This includes voice platforms (Retell, ElevenLabs), telephony (Twilio), cloud infrastructure (AWS/Azure), EMR integration partners, and any analytics or monitoring tools that may access patient data.

- Gate all vendor onboarding through BAA review before any PHI flows.
- Maintain a centralized BAA repository with version control and renewal alerts.
- Include right-to-audit, breach cooperation, and subcontractor flow-down clauses.
- Ensure product and engineering teams know which environments are in-scope for BAA data.

Data Handling Standards

- Minimum necessary use: Only access the PHI you need for the specific task.
- Secure communication: No consumer messaging apps for PHI. Use HIPAA-compliant platforms.
- Device security: Screen locks, encryption, and cable locks on all workstations.
- Data disposal: Secure destruction procedures for any media containing PHI.
- Access reviews: Quarterly review of who has access to what. Revoke immediately upon role change.

Incident Response Protocol

Step	Action	Timeline	Owner
1. Detect	Identify potential breach or security incident	Immediate	Any team member
2. Report	Escalate to Security Officer and document	Within 1 hour	Security Officer
3. Contain	Isolate affected systems, preserve evidence	Within 4 hours	Tech Lead
4. Investigate	Determine scope, affected data, root cause	Within 24 hours	Security Officer + Tech Lead
5. Notify	Notify affected individuals and regulators as required	Per HIPAA deadlines	CEO + Privacy Officer
6. Remediate	Fix root cause, update controls, document lessons	Within 1 week	Tech Lead

9. KPI and Success Metrics

Pilot KPI Dashboard

Every metric feeds back to the founding thesis: we help clinics run smoother, capture more revenue, and reduce admin burden.

A. Call Handling Efficiency

KPI	Baseline	Month 1 Target	Month 3 Target
Calls Answered by AI (%)	0%	40-50%	70-80%
Missed Calls Reduction	Baseline count	-40%	-70%
Average Answer Time	Baseline	Under 10 sec	Under 5 sec
AI Task Completion Rate	-	85%	95%+
Human Handoff Rate	-	15-20%	5-10%

B. Intake Optimization

KPI	Baseline	Month 1 Target	Month 3 Target
Pre-Visit Intake Completion	0%	30%	60-70%
Avg Intake Time (min)	12-15	8-10	4-6
Data Accuracy Improvement	Baseline error %	-50%	-80%
Intake Staff Hours/Week	Baseline	-20%	-40%

C. Appointment and Revenue

KPI	Baseline	Month 1 Target	Month 3 Target
No-Show Rate (%)	15-20%	10-12%	6-8%
Appointment Slots Filled	Baseline	+8%	+15%
New Patient Capture Rate	Baseline	+5%	+12%
Revenue Protected/Week	\$0	\$500-1,000	\$1,500-2,500

D. Financial ROI Summary

Metric	Month 1	Month 3	Notes
Implementation Cost	\$3,000-5,000	\$5,000-8,000	One-time setup
Monthly Recurring Cost	\$500-800	\$500-800	Platform + support
Staff Labor Savings/Mo	\$1,500-2,500	\$3,000-5,000	Hours freed
Revenue Protected/Mo	\$800-1,500	\$2,000-3,500	No-shows prevented
Efficiency Gains/Mo	\$1,000-1,500	\$2,500-4,000	Capacity boost
Total Monthly Benefit	\$3,300-5,500	\$7,500-12,500	Combined savings
Payback Period	1-2 months	Under 1 month	Positive ROI
3-Month Net Benefit	-	+\$15K-30K	Cumulative

Weekly Tracking Format

Every Monday at 8 AM, the operations manager sends the weekly dashboard. Format:

- Green Flags: Metrics meeting or exceeding targets.
- Yellow Flags: Metrics within 10% of target, with explanation and fix in progress.
- Red Flags: Metrics significantly below target, with root cause and remediation plan.
- Financial Snapshot: Monthly savings, revenue protected, net benefit for the week.
- Next Week Focus: Top 3 priorities for the coming week.

Success Milestones

Month 1 Victory: AI answering 40-50% of calls. No-shows reduced by 25%. Front-desk team reports improvement. Zero critical system failures.

Month 3 Victory: AI handling 70-80% of calls. No-show rate at 8-10%. \$15,000-30,000 net positive. Ready to sell playbook to second clinic.

Month 6 Victory: System runs like infrastructure. Repeatable playbook documented. 2-3 clinics operational. Word-of-mouth generating inbound interest.

10. Team Accountability

Role Ownership Map

Role	KPI Ownership	Cadence
Dr. Nikhil (CEO)	Overall ROI, clinical workflows, brand narrative, sales	Weekly review
Operations Manager	Call handling, appointments, revenue metrics	Weekly dashboard
Front-Desk Lead	AI accuracy, staff burden, patient experience	Daily check-in + weekly report
Technical Lead	System uptime, integrations, data sync, security	Daily monitoring + weekly report
Finance/Admin	Cost tracking, payback calculation, labor savings	Weekly

Weekly Cadence

- Monday 8 AM: Weekly KPI dashboard sent to all stakeholders.
- Monday 9 AM: 30-minute review call: what worked, what did not, priorities for the week.
- Daily: 15-minute standup (tech team): blockers, progress, next steps.
- Daily: Front-desk lead checks AI accuracy and flags issues.
- Friday 5 PM: Technical lead sends system health report (uptime, errors, sync status).

Escalation Protocol

Severity	Example	Response Time	Escalation Path
Critical	System down, patient data exposed	Immediate	Tech Lead then CEO within 15 min
High	AI accuracy drops below 85%	Within 2 hours	Tech Lead then Ops Manager
Medium	Single device sync failure, minor bug	Within 24 hours	Tech Lead
Low	Feature request, minor UI issue	Within 1 week	Ops Manager logs for sprint planning

11. The Do's and Don'ts

These are the guardrails. Violating a "don't" is not a mistake. It is a failure of discipline. Following a "do" is not optional. It is baseline expectation.

Product Do's and Don'ts

Do:

- Test every feature in your own clinic before shipping to any customer.
- Build human handoff into every AI interaction. The patient must always be able to reach a person.
- Start with the simplest version that solves the problem. Add complexity only when proven needed.
- Collect baseline data before turning on any system. You need before-and-after proof.
- Design call flows based on real front-desk conversations, not assumptions.
- Roll out gradually: after-hours first, then overflow, then full integration.
- Update training data regularly. What works at launch may drift in 3 months.
- Build a feedback loop: when AI gives a wrong answer, that triggers a content review.
- Track which questions the AI struggles with and fill those gaps weekly.
- Integrate with the EMR and scheduling system. A standalone bot is a toy, not a product.

Don't:

- Don't build the voice engine from scratch. Use Retell, ElevenLabs, or OpenAI APIs. Your value is the workflow, not the model.
- Don't launch at full volume on day one. A bad first impression with patients is nearly impossible to recover from.
- Don't try to handle every call type from the start. Start with scheduling and confirmations. Expand from there.
- Don't let AI responses go to patients without a quality control step during the first 2 weeks.
- Don't ignore the conversation after it happens. Review transcripts, learn, improve.
- Don't build features no clinic has asked for. Talk to clinics first. Build second.
- Don't make the voice bot sound robotic. Use conversational, plain language. Patients know it is AI. Make it feel warm anyway.
- Don't skip emergency escalation triggers. Keywords like 'chest pain' or 'can't breathe' must route to a human instantly.

Sales Do's and Don'ts

Do:

- Lead with Dr. Nikhil's story. His credibility is the most powerful sales tool you have.
- Sell outcomes: 'zero missed calls,' 'automatically filled slots,' 'fewer angry patients.'
- Offer a risk-free pilot. Remove every possible reason for a clinic to say no.
- Quantify the pain in dollars. Show them what missed calls and no-shows cost per month.
- Share real numbers from your own clinic. Proof beats promises every time.
- Understand their specific workflow before proposing anything. Spend time in their front desk environment.
- Talk to the office manager, not just the doctor. The office manager is the daily user and the real buyer.
- Follow up every verbal conversation with a written summary and next steps.
- Be honest about what the AI cannot do. Under-promise, over-deliver.

Don't:

- Don't pitch 'AI' as the product. Clinics are tired of AI hype. Pitch operational results.
- Don't exaggerate implementation timelines or capabilities. Broken promises kill trust permanently.
- Don't try to replace the front desk. Position AI as their assistant, not their replacement.
- Don't ignore the budget cycle. Understand when clinics make purchasing decisions and plan around it.
- Don't offer enterprise-level customization at this stage. Stay focused on the repeatable playbook.
- Don't use jargon: NLU, voice synthesis, API integration. Say: 'answers calls,' 'books appointments,' 'syncs with your system.'
- Don't ask clinics to do technical setup. You handle everything. Zero burden on the clinic is non-negotiable.
- Don't try to compete with hospital-grade systems. Own the independent clinic market. Stay focused.

Compliance Do's and Don'ts

Do:

- Sign BAAs with every vendor before any PHI flows. No exceptions.
- Encrypt all patient data in transit and at rest. Use AES-256 and TLS 1.2+.
- Use unique IDs with MFA for all team members and system access.
- Conduct annual risk assessments plus ad-hoc assessments after major changes.
- Train every team member on HIPAA within their first week. Refresh annually.
- Maintain audit logs for all data access, authentication, and admin actions.
- Use HIPAA-compliant communication channels. No WhatsApp, iMessage, or SMS for PHI.
- Document everything: policies, risk assessments, training records, incident reports.
- Run access reviews quarterly. Revoke access the same day someone leaves or changes roles.
- Test your incident response plan before you need it.

Don't:

- Don't treat HIPAA as a marketing badge. It is a legal and ethical obligation.
- Don't assume cloud provider compliance equals application compliance. AWS being HIPAA-ready does not make your app HIPAA-ready.
- Don't store real PHI in test or staging environments. Use synthetic or de-identified data.
- Don't skip the BAA because a vendor says they are 'HIPAA-compliant.' Get it in writing.
- Don't let anyone access more PHI than they need for their specific role. Minimum necessary, always.
- Don't wait to report a suspected breach. Report within 24 hours internally. Every time.
- Don't use consumer apps (Slack free tier, Google personal accounts, Dropbox basic) for patient data.
- Don't rely on point-in-time compliance checks. Build continuous monitoring and alerting.

Culture Do's and Don'ts

Do:

- Document everything you learn. If a workflow works, write it down. If it fails, write it down louder.
- Share bad news faster than good news. Hiding problems makes them worse.
- Own your mistakes publicly. Blame culture kills startups.
- Treat clinic staff with the same respect you give their doctors. They are your daily users.
- Celebrate outcomes, not effort. Shipped results matter more than long hours.
- Hire people who care about healthcare operations, not just people who are good at coding.
- Keep the team small and accountable. Every person should own a KPI.
- Default to async communication. Protect deep work time.
- Run 90-day planning cycles. Long-term plans are guesses. 90-day plans are commitments.

Don't:

- Don't hire for pedigree. Hire for hustle, adaptability, and genuine interest in the problem.
- Don't tolerate 'that is not my job.' In a startup, everything is everyone's job.
- Don't let meetings replace work. If it can be an async update, make it one.
- Don't skip the weekly review. It is the single most important meeting. Non-negotiable.
- Don't let success make you complacent. Month 1 working does not mean Month 6 will be easy.
- Don't scale the team before scaling the playbook. Process first, people second.
- Don't lose sight of why you started: making clinics run better, not building cool technology.

Technical Do's and Don'ts

Do:

- Use Retell or ElevenLabs for the voice engine. Your value is in the workflow layer, not the speech model.
- Build deep integrations with clinic scheduling and EMR systems (Athenahealth, Practo, etc.).
- Target 99.5% uptime in Month 1 and 99.9% by Month 3. Clinics depend on this 24/7.
- Implement real-time monitoring with automated alerts for failures.
- Use standardized data formats (HL7 FHIR) for EMR integration wherever possible.
- Design for graceful degradation. If the AI fails, the call routes to voicemail or human, never drops.
- Run 50+ test calls across all scenarios before any clinic goes live.
- Separate production from staging environments completely. No shortcuts.
- Log every AI interaction for review. These logs drive the next version of the playbook.
- Build patient-friendly scripts. The AI should sound helpful, not corporate.

Don't:

- Don't build a voice engine from scratch. That is a billion-dollar R&D; project. You are not Anthropic.
- Don't optimize for voice latency at the expense of workflow accuracy. Getting the answer right matters more than getting it fast.
- Don't hardcode clinic-specific values. Build configurable call flows that work across multiple clinics.
- Don't skip load testing. If 5 patients call at the same time, the system must handle it.
- Don't deploy on Friday. Ever. If something breaks, you lose the weekend.
- Don't ignore EMR sync failures. A single lost data point can delay patient care.
- Don't use consumer-grade infrastructure. Use HIPAA-ready cloud services from day one.
- Don't neglect the dashboard. If clinics cannot see the value, they will assume there is none.

This Is Your Operating Truth

This playbook is not about the AI being perfect. It is about the clinic running better.

Every KPI, every process, every rule in this document points to one question:

Are we making this clinic's life easier and more profitable?

Track religiously. Adjust weekly. Build the playbook by doing the work.

Sources and References

1. KLAS Research, "Playbook for Patient-Centric Care 2025," <https://klasresearch.com/report/klas-playbook-for-patient-centric-care-2025-how-to-succeed-in-patient-centricity/3836>
2. Accountable HQ, "HIPAA Compliance for Startups," <https://www.accountablehq.com/post/how-health-tech-startups-can-achieve-hipaa-compliance-requirements-checklist-and-best-practices>
3. Curogram, "Clinic Front Desk Workflow Automation," <https://curogram.com/blog/clinic-front-desk-workflow-automation>
4. MedLaunch, "Complete Guide to Automate Front Desk Calls in Healthcare," <https://medlaunch.health/blogs/practice-growth/automate-front-desk-calls-guide/>
5. Vanta, "HIPAA for Healthtech Companies," <https://www.vanta.com/collection/hipaa/hipaa-for-healthtech-companies>
6. Konfirmity, "HIPAA Dos and Dents 2026," <https://www.konfirmity.com/blog/hipaa-dos-and-dents>
7. Healthtech Off The Record, "How to Sell AI to Doctors," <https://healthtechofftherecord.substack.com/p/how-to-sell-an-ai-product-to-doctors>
8. The Healthcare Blog, "9 Things Every Healthcare Startup Should Know," <https://thehealthcareblog.com/blog/2020/01/15/9-things-every-healthcare-startup-should-know-about-business-development/>